

Managing an acutely-disturbed person

Key steps if someone presents in primary care
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Red Whale

GEMS
Guidelines & Evidence Made Simple

Patient management

- Keep a calm external demeanour.
- Try to engage with the patient – ask their name.
- Listen to the patient and show you are listening:
 - Summarise and reflect back what they are saying.
 - Pay attention to any cues.
 - Reassure that you want to help.
- Find a safe, quiet place for them to sit. Offer them a drink. Have someone stay with them (if safe).

Manage risks

- **Raise the alarm**, preferably without causing alarm (use the silent panic button).
 - **Consider risks to staff and patients:** does (part of) the building need to be evacuated?
 - **Should the police be called** (ideally without the patient knowing as this may exacerbate things)?
- Early risk management may prevent the situation deteriorating, or, if it does, the appropriate people will be available to manage it.**

Clinical assessment

- **Get a summary of the medical records to the assessing clinician.** Do they have a crisis plan in their notes?
- **Involve any staff who already know the patient.** This may help you in your assessment, and seeing someone familiar may help calm the patient too.
- **Is there a relative/friend available who can give a collateral history?** If the patient specifically refuses permission to speak to relative/friend, assess whether the patient has capacity to make that decision. If they don't, you may need to make a 'best interest' decision to speak to the informant.
- **Consider the differential diagnosis** and ask the relevant questions to explore this.
- **Ask about past medical history and current problems.**
- **Ask about drugs (prescribed or otherwise) and alcohol.**
- **Assess risk of harm to patient or others.** Risk factors include:
 - Recent discharge from inpatient care.
 - Current drug/alcohol misuse.
 - History of self-harm or suicide.
 - Relevant past forensic history, for example assaults.
 - History of carrying weapons.
 - Delusions that focus on a specific individual.

Differential diagnoses

- **Stress reaction** (relationship breakdown, loss of job/housing/financial crisis)
- **Intoxication/withdrawal from drugs/alcohol**
- **Psychosis** (first or subsequent episode)
- **Organic causes** (drug reaction, infection, neurological, metabolic condition)
- **Exacerbating psychiatric comorbidities** (e.g. personality disorder + acute stress reaction)

Refer on!

- If patient accepts referral:** decide on most appropriate place (depends on how local services are organised).
- If they are known to services and have a crisis plan, contact the care coordinator.
 - Urgent psychiatric assessment may be needed via local psychiatric services.
 - If physical health concerns (alcohol withdrawal, physical illness suspected): A&E may be safest place.

If the patient declines referral:

- **Immediate risk to self or others:** police can detain under section 136 and remove patient to place of safety (usually a psychiatric hospital), for the purpose of an assessment (the only place this section can't be used is in a private house or garden).
- **No immediate risk to self/others and the patient has calmed down:** arrange a Mental Health Act Assessment, if appropriate. Usually not immediately, so patient must be safe to go home.

Debrief

- Have a team debrief: identify what went well and where there were gaps/things could be improved.
- For non-clinical staff, this can be a shocking event – make sure they have the support they need.
- Contact the patient on discharge and arrange a review of mental health, relapse indicators, crisis plan and local support. People with mental health problems have significantly worse physical health outcomes so use this as an opportunity to review their physical health too.

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